

Reproductive Health

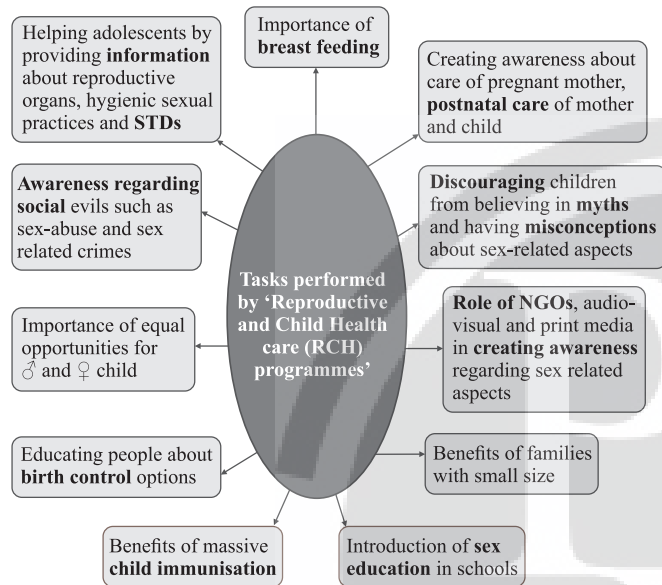
SHORT NOTES

Introduction

- ❖ According to WHO, reproductive health means a total well being in all aspects of reproduction i.e., physical, emotional, social and behavioural.

Problems and Strategies

- ❖ India was amongst first country in the world to initiate action plans at a national level to attain total reproductive health such as family planning programmes in 1951.



Amniocentesis

- ❖ Analyse foetal cells and dissolved substances from amniotic fluids.
- ❖ Technique used to check for genetic disorders such as Down's syndrome, sickle-cell anemia, etc.
- ❖ Statutory ban on this technique in India to prevent female foeticide.

Population Stabilisation

- ❖ According to 2001 census, our population growth rate was around 1.7 per cent, i.e., 17/1000/year.

Year POPn	1900	1947	2000	2011
World	2 billion		6 billion	7.2 billion
India		approx: 350 million	close to 1 billion	crossed 1.2 billion

Reasons for Increase in Population Size:

- ❖ Decline in death rate
- ❖ Rapid decline in maternal mortality rate (MMR)
- ❖ Decrease in infant mortality rate (IMR)
- ❖ Increase in number of people in reproductive age
- ❖ Increase in health facilities

Measures Taken by Government to Check Population

- ❖ Motivate smaller families for using various contraceptive methods and by slogans "Hum do Hamare do", in advertisements and posters.
- ❖ Urban couples adopting: "One child norm".
- ❖ Statutory raising of marriageable age:
 - + Female- 18 years
 - + Male- 21 years
- ❖ Incentives given to couples with small families.

Birth Control/Contraception

Features of an Ideal Contraceptive:

- ❖ User-friendly
- ❖ Easily available
- ❖ Effective
- ❖ Reversible
- ❖ No/least side-effects
- ❖ No interference with libido or act of coitus

Two Principle Methods of Birth Control:

- ❖ Natural methods
- ❖ Artificial methods

Natural/Traditional Methods

- ❖ Periodic abstinence
- ❖ Withdrawal method/Coitus interruptus
- ❖ Lactational amenorrhea
- ❖ Based on the principle of avoiding physical meeting of the egg and sperms
- ❖ Chances of failure are high

Artificial Methods

- ❖ Barrier methods
- ❖ Spermicidal jellies
- ❖ IUDs
- ❖ Oral pills
- ❖ Injections and implants
- ❖ Emergency contraceptives
- ❖ Surgical methods

Medical Termination of Pregnancy (MTP)/Induced Abortion

- ❖ **MTP:** Intentional or voluntary termination of pregnancy before full term.
- ❖ MTP was legalized in India in 1971.
- ❖ 40–50 million MTPs are performed every year.
- ❖ 1/5th of the total number of conceived pregnancies.

- ❖ Intention behind MTP amendment act 2017, (Government of India)
 - + Reducing the incidence of illegal abortion.
 - + Decrease consequent maternal mortality and morbidity.
 - + MTPs are safe upto 12 weeks but riskier in 2nd trimester yet both are legal.
 - + Amniocentesis and MTPs have been misused in context of female foeticide.

Sexually Transmitted Diseases (STDs)

- ❖ **Alternately named:** Venereal diseases (VD) or reproductive tract infections (RTIs).
- ❖ **High vulnerability/risk group:** 15-24 years.
- ❖ Bacterial and protozoan diseases are completely curable if detected early and treated properly.
- ❖ **Mode of Transmission:**
 - + Sexual intercourse.
 - + Sharing of injection needles, surgical instruments with infected persons.
 - + Transfusion of blood.
 - + From infected mother to foetus.

Preventive measures to avoid STDs:

- + Avoid sex with unknown partners/multiple partners
- + Always try to use condoms during coitus

Infertility

- ❖ Unable to produce children inspite of unprotected sexual co-habitation.
- ❖ Infertility as a problem could be with either the male or female partner.
- ❖ In India, female is blamed often than male for the couple being childless.

Assisted Reproductive Technologies (ART)

<i>In-vitro</i> fertilization	<i>In-vivo</i> fertilization
Outside the body in simulated conditions in laboratory	In the female reproductive tract
Yes	No
ZIFT, IUT, ICSI	GIFT, AI, IUI